Reese Spine & Injury Center

18188 Rebecca Keys Suite 231
Pearland, TX
Phone 832-555-3292

Itemized Medical Bill

Patient:	Ryan Little
Date of birth:	1958-02-04
Insurance claim:	C-1429
Date of injury:	2025-05-26
First visit:	2025-06-02
Last visit:	2025-08-03
Number of visits:	10

Services

Service	Units	Charge
99203 New patient evaluation	1	$347.19
98941 Spinal manipulation	2	$238.30
97110 Therapeutic exercise	3	$506.07
72040 X-ray, cervical spine	2	$1,778.76
20552 Trigger point injection	2	$1,060.06
TOTAL		$3,930.38

Recent visits

Date	Service	Charge
2025-06-30	20552 Trigger point injection	$560.22
2025-07-10	72040 X-ray, cervical spine	$632.47
2025-07-15	20552 Trigger point injection	$499.84
2025-07-19	97110 Therapeutic exercise	$150.10
2025-07-29	72040 X-ray, cervical spine	$1,146.29
2025-08-03	97110 Therapeutic exercise	$187.59

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Treatment History and Summary

Patient:	Ryan Little
Treating provider:	Timothy York, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
June 2025: chiropractic adjustment, evaluation, pain management, physical therapy	5	$1,314.09
July 2025: imaging, pain management, physical therapy	4	$2,428.70
August 2025: physical therapy	1	$187.59

Treatment summary

Patient presented following a low-speed head-on motor vehicle collision with complanits of rihgt knee pain, swelling, and reduced range of motion. The reported mechanism of injury is consistent with the force of the collision and the pattern of symptoms described. No prior injury or treatment involving this body area is known. Over the course of ten visits, care followed a conventional progression, beginning with rest, ice, anti-inflammatory medication, and guided physical therapy. Symptoms improved steadily, and visit frequency was reduced accordingly as function returned. Examination findings were mixed: effusion and tenderness along the medial joint line support the reported injury, while ligament stability testing and neurovascular assessment were within normal limits. Billed services correspond to the clinical care documented, with no duplicate or repeated chagres identified. At last visit, the patient was improving, with residual stiffness expected to resolve with a home exercise program. Continued follow-up is advised only if symptoms recur or fail to progress.